



DaVita Renal Center Of Succasunna

175 Righter Rd, Succasunna, NJ, 07876 · DaVita · CCN 312623 · September 22, 2026

OPERATOR / TENANT DaVita	ASSET TYPE Single-tenant medical 4,704 SF	CAPACITY UTILIZATION 76.5%	EST. ANNUAL REVENUE \$2.61M
EST. OPERATING MARGIN 20.9%	DIALYSIS STATIONS 12 chairs	CMS STAR RATING ★★★★★	CENSUS TREND Stable

At a glance — A 12-station DaVita in-center hemodialysis facility running at 76.5% of modeled operating capacity. Corrected facility revenue is \$2,613,271 with 6,169 CMS annual treatments, with a commercial/private payer share of 47.1%.

TENANT & OPERATOR

Source: CMS · operator SEC filings

Operator	DaVita
Service Modality	In-center hemodialysis
Dialysis Stations	12

FACILITY FINANCIAL PERFORMANCE

Source: CMS/HCRIS · operator filings

Est. Facility Revenue (annual)	\$2,613,271 Corrected clinic economics.
Revenue / Treatment (blended)	\$424 Weighted by this facility's payer mix.
Est. Operating Profit	\$545,538
Operating Margin	20.9% Derived from corrected clinic economics.
Annual Treatments	6,169 CMS medicare_clinics TTM treatments.

DEMAND & CAPACITY

TREATMENT DEMAND		CATCHMENT CONTEXT	
Dialysis Stations	12	CMS Reported Patients (annual)	184
Patients in Treatment (concurrent)	46		Cumulative distinct patients per CMS reporting — not a point-in-time count (see definitions).
Patients / Chair	3.8	Census Trend	Stable (+0.0% over 3 yr)
Capacity Utilization	76.5%		
Treatments / Year	6,169		

Reconciling the two patient figures — The facility treats roughly **46 patients at a time** — the figure that reconciles with chairs, utilization, and annual treatments. CMS separately reports **184** distinct patients served over its annual reporting window, a cumulative count (new starts, transfers, short-stay) shown here as catchment depth, not chair load.

HOURS OF OPERATION

Monday	6:45 AM – 5:00 PM
Tuesday	7:00 AM – 5:00 PM
Wednesday	6:45 AM – 5:00 PM
Thursday	7:00 AM – 5:00 PM
Friday	6:45 AM – 5:00 PM
Saturday	7:00 AM – 5:00 PM
Sunday	Closed

61 hours/week

QUALITY METRICS

CMS Star Rating	5 / 5
Mortality Rate	20.9 (national avg: 15.0)
Hospitalization Rate	109.1 (national avg: 150.0)
Readmission Rate	28.7 (national avg: 25.0)
Infection Ratio	N/A

Deficiency Count	N/A
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PAYER MIX

Medicare	48.8%
Medicaid	4.1%
Commercial / Private	47.1%

COMPARATIVE BENCHMARKING

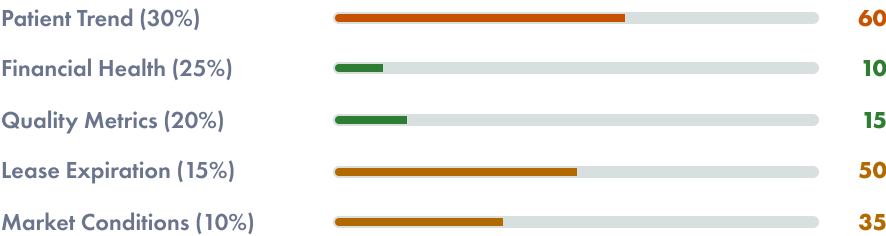
Source: CMS • 7,670 U.S. facilities

Percentile shows where this facility ranks among peers (higher = larger).



RISK ASSESSMENT

Composite lease-renewal model • 0 (low) – 100 (high)



Lower component scores indicate lower risk; see definitions for inputs.

MARKET POSITION

Source: CMS Provider of Services

In-County Competition

7 other facilities in Morris County

Nearest Facility	City	Distance	Operator	Chairs	Patients	Rent PSF
677 Route 46 Suite C	Kenvil, NJ	1.6 mi	Fresenius	20	110	—
400 West Blackwell St	Dover, NJ	4.1 mi	Fresenius	13	44	—
417 Us Highway 46	Dover, NJ	5.9 mi	DaVita	12	—	—
135 Kendall Ct	Rockaway, NJ	6.2 mi	Independent	—	—	—
2200 Route 10 West	Parsippany, NJ	8.9 mi	Fresenius	21	89	—

Rent / PSF is shown where a confirmed lease comp exists.

PROPERTY DETAILS

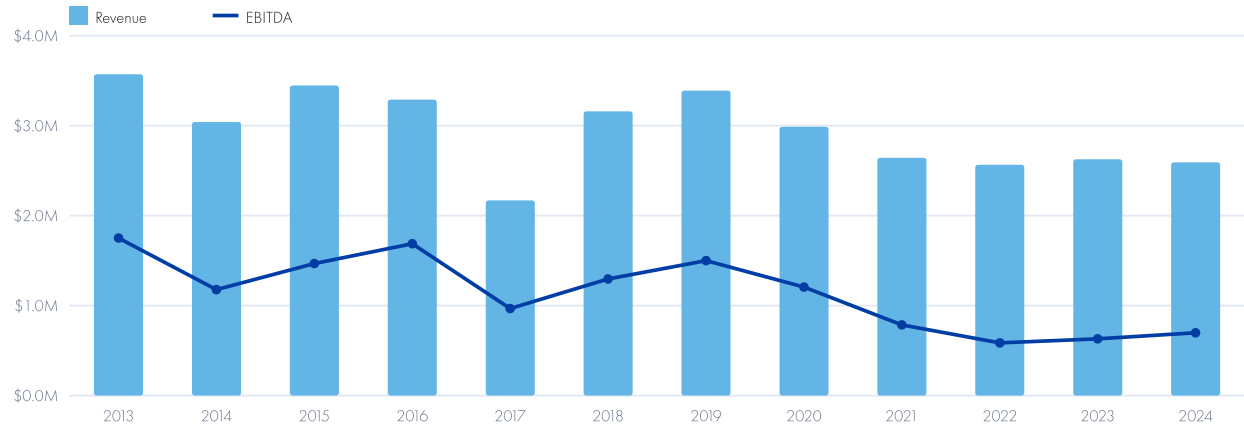
Address	175 Righter Rd, Succasunna, NJ, 07876
Building Size	4,704 SF
Land Area	0.57 acres

Property Type	Medical Office – Dialysis Clinic
Year Built	1960 (renovated 2025)

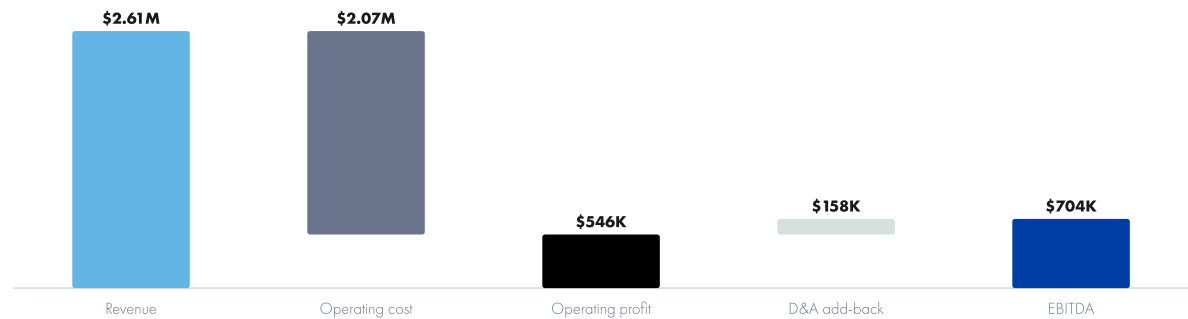
FACILITY FINANCIAL PERFORMANCE

Reconciled per-year economics (model dialysis_econ_reconciled_v1) — HCRIS-anchored volume & cost, payer-mix-weighted revenue, validated against operator 10-K filings · high confidence. Trend is volume-driven (rates held at CY2024).

Revenue & EBITDA over time



How the number is built — FY2024



Real-estate value crosswalk — rent coverage

EBITDA (net of rent)	\$704K
+ Contract rent	\$172K
= EBITDAR (rent coverage basis)	\$876K
Rent coverage	5.09×
Rent as % of EBITDAR	19.6%

Implied real-estate value = contract rent ÷ cap rate

6.00%	6.50%	7.00%	7.50%
\$2.87M	\$2.65M	\$2.46M	\$2.29M

Coverage of 5.09× means facility cash flow (before rent) covers the rent 5.1 times over — a measure of tenant durability supporting the cap rate.

Methodology & Data Sources

Revenue & Operating Profit (Reconciled Truth Model)

Revenue and operating profit are a single reconciled figure per facility per fiscal year (model *dialysis_econ_reconciled_v1*), rebuilt from first principles rather than a modeled-estimate-vs-actual either/or. The audited CMS HCRIS cost report is treated as what it actually is — a cost report (it books allowable cost as “revenue,” so revenue equals cost on every filing) — and therefore as a reliable anchor for **treatment volume** (Worksheet S) and **operating cost** (Worksheet D), but not for top-line revenue. Revenue is reconstructed as **treatments ×**

a **payer-mix-weighted reimbursement rate** (CY2024 schedules: Medicare ~\$279, Medicaid ~\$225, Commercial ~\$1,100, Other ~\$250/treatment); operating profit = revenue – HCRIS cost. Because a facility's reported private-payer share conflates Medicare Advantage (which pays roughly Medicare rates) with true commercial, only a calibrated fraction of that share is priced at the commercial rate — calibrated so the national blend reproduces the observed sector operating margin (~16%). Legacy internal estimates are blended in only when they fall within a plausibility band of the payer-weighted rate; implausible values are excluded. Each facility carries a confidence tier (high/medium/low) based on the strength of its underlying HCRIS and payer-mix data.

Validation against public filings

Aggregated bottom-up across all ~8,300 facilities, the model reconciles to operator SEC filings: for DaVita facilities the implied revenue-per-treatment (~\$380) matches DaVita's FY2024 10-K (~\$369–380/treatment) within ~3%, and the facility-level EBITDA margin (~25%) aligns with DaVita's reported dialysis EBITDA. Aggregate revenue is conservative relative to operator-reported totals (our treatment counts are audited CMS figures, which run below operator-reported treatments that include hospital/acute settings) — the model does not overstate.

EBITDA

EBITDA = operating profit + estimated depreciation & amortization. D&A is anchored to operator 10-K disclosures (~\$27/treatment) and distributed to each facility by **size** (treatment volume) and **age** (a plant/equipment depreciation curve from the facility's CMS certification age), bounded to a sane share of revenue. EBITDA is reported only where operating profit is available; it is never a fabricated add-back.

Treatment Volume

Annual treatment counts are anchored to HCRIS cost-report actuals (Worksheet S); where a facility has no HCRIS filing, volume falls back to the CMS trailing-twelve-month total, then to patient census × 156 (3× weekly, 52 weeks). The app's treatment counts reconcile to HCRIS at a median ratio of 1.00 across ~7,500 facilities.

Capacity Utilization

Capacity Utilization is a modeled estimate of treatment volume against the facility's estimated operating capacity (stations × shifts × operating days × a standard utilization factor) — *not* a real-time occupancy reading. A figure near 90% indicates the facility is modeled to run close to its practical chair throughput; it does not mean that share of chairs is filled at any single moment.

Patient Counts

"Patients in Treatment (concurrent)" is the estimated point-in-time census (HCRIS-derived) and is the figure that reconciles with chairs, utilization, and annual treatments. "CMS Reported Patients (annual)" is the cumulative count of distinct patients served over CMS's reporting window and is typically several times larger; it is shown as catchment depth only.

Comparative Benchmarking

Percentile rank among CMS-certified dialysis facilities by estimated patient volume and revenue, within the state and nationally (higher percentile = larger facility). County-level ranking is omitted where the facility is the only one in its county.

Operating Costs & Margins

Operating cost is the audited HCRIS facility cost-report total (Worksheet D, all-payer, the same figure that anchors the reconciled model above); the median is ~\$303/treatment and falls with facility scale (~\$428/treatment at the smallest facilities to ~\$278 at the largest). Margin is operating profit divided by revenue. Because cost is the audited per-facility figure rather than a flat assumption, genuinely sub-scale facilities can show a thin or negative operating margin — a real, audited signal, not a data gap.

Revenue & Profit Trend

Year-over-year and multi-year CAGR are computed from the facility's reconciled per-year series (FY2011–present), applying one figure per fiscal year with an order-of-magnitude outlier guard so definition mismatches cannot corrupt the trend. Reimbursement rates are held at the current (CY2024) schedule across all years, so the trend is **volume-driven** — it reflects the facility's treatment-volume trajectory (the

signal that most drives value), not rate inflation.

Quality & Risk Metrics

Quality data sourced from CMS Dialysis Facility Compare. Mortality, hospitalization, and readmission rates are per 100 patient-years, benchmarked against national averages (mortality ~15, hospitalization ~150, readmission ~25). The composite lease renewal risk score (0–100) weights five factors: Patient Trend (30%), Financial Health (25%), Quality Metrics (20%), Lease Expiration (15%), and Market Conditions (10%).

Competitive Landscape

Competitor data includes all CMS-certified dialysis facilities in the same county, sourced from the CMS Provider of Services file. Patient counts reflect the most recent CMS enrollment snapshot. Geocoded distance-based analysis will be available in a future release.

Data Sources

CMS Dialysis Facility Compare, CMS HCRIS Cost Reports, CMS Provider of Services, USRDS Annual Data Report, MedPAC Payment Reports, DaVita/Fresenius 10-K SEC Filings, U.S. Census ACS Demographics, CDC PLACES Health Data, Google Places API (hours of operation).